



OFFICIAL

Mental Health Seclusion Policy

1. Applicability

Site	Operational Area	Applicable to
Armadale Health Service	Mental Health Service	All workforce groups

2. Purpose

The purpose of this policy is to describe safe clinical and documentation requirements should seclusion be required to ensure the safety of the mental health consumer, staff or others within the Armadale Health Service Mental Health Unit (MHU) in accordance with the *Mental Health Act 2014* (section 215-224), the [Chief Psychiatrist's Standards for Clinical Care](#) and the [EMHS Reducing Restrictive Practices in Mental Health Services Policy](#).

3. Scope

This policy applies for the authorised Mental Health Unit (MHU) at Armadale Health Service and all healthcare workers that work within the MHU.

This policy does not apply to the following:

- Persons who are not mental health consumers of the health service (e.g. visitors, relatives and friends)
- Persons under arrest or a prisoner of WA Police or Department of Correctional Services where statutory requirements exist and obligation to public safety and maintaining custody, override medical need
- Discharge against medical advice patients – refer to the [EMHS Discharge Against Medical Advice Policy](#).

4. General principles

The following principles are applicable to the use of seclusion in the MHU:

- The use of seclusion is not considered a therapeutic intervention and should be avoided wherever possible.
- Seclusion may only be considered when there is an imminent risk of danger to the individual or others and no other safer intervention is identified at the time.
- Seclusion is only to be used for a minimum period and must be terminated as soon as it is safe to do so.
- Where there are no appropriate alternatives to seclusion, it is to be administered in the safest, dignified, compassionate and respectful manner as possible by appropriately trained staff and for the shortest possible duration.

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- The use of seclusion creates significant risks for people with mental illness, their carers and staff. These risks may include serious injury, physical harm and /or psychological trauma, loss of dignity, cultural harm and breakdown in therapeutic relationships.
- The mental health consumer must be examined by a medical practitioner within two (2) hours of seclusion commencing.
- The mental health consumer, and with consent, their family/carer or nominated Personal Support Person (PSP), will be actively involved in the assessment process and their ongoing care planning and treatment choices.

5. Pre-Seclusion

- On admission, clinical staff will ensure a risk assessment is undertaken to identify any potential for aggression or violence by the mental health consumer.
- A Psychiatric Services Online Information System (PSOLIS) Treatment Support and Discharge Plan will be completed with the mental health consumer and carer/family member as soon as practicable on admission.

In addition, a Consumer Safety Plan will be developed in accordance to the [Consumer Safety Planning Procedure](#). This will outline precipitating and exacerbating factors which may result in seclusion and outline a graded series of responses.

- Medical Practitioners, mental health practitioners or the person in charge of the ward must explore all least restrictive interventions prior to restraint being considered. Prevention and early interventions to consider include, but are not limited to:
 - Implementation of the mental health consumer and/or family/carer's documented advice regarding the appropriate strategy to reduce distress and agitation
 - Defusing and de-escalation techniques
 - Peer support involvement, dependent upon consent of the mental health consumer and availability of the Peer Support Worker
 - Ensuring a culturally secure environment. This may include, if safe and appropriate, consultation with Aboriginal Mental Health Workers to communicate mental health consumer's options
 - Diversional activities and sensory modulation
 - Low stimulus environment (e.g. comfort room, outdoor area, dedicated low stimulus zone)
 - PRN medication, used in conjunction with other strategies, for the purpose of calming, not sedating.

Seclusion exclusion criteria

- Seclusion may not be used for the purposes of discipline, coercion or staff convenience such as managing inadequate staffing levels.
- As seclusion has been shown not to be particularly effective in preventing imminent self-harming behaviours, seclusion should not be used for this sole purpose. Other strategies to manage self-harming behaviours should be employed.

6. Authorising seclusion

A medical practitioner, mental health practitioner, or the person in charge of a ward may make a seclusion order (in accordance with MHA 2014 s. 214) authorising the seclusion of:

- A person who is a mental health consumer admitted to the authorised inpatient unit
- A person who is referred for examination by a psychiatrist at the authorised inpatient unit
- A child who is under an order to enable an examination to be conducted by a psychiatrist at the authorised inpatient unit.

If seclusion is required urgently, either a medical practitioner, mental health practitioner or the person in charge of the ward can make an [Record of Oral Authorisation of Seclusion Form \(Form 11A\)](#) or [Written Seclusion Order \(Form 11B\)](#) for the person to be placed into seclusion:

- As soon as practicable after an oral seclusion order is authorised, the staff member who made the authorisation must record the use of seclusion on the Form 11A.
- For those mental health consumers who may be physically or medically compromised (e.g. pregnancy, heavy sedation, active medical illnesses etc.) a medical practitioner is required to document, as soon as possible after seclusion is commenced, using Form 11B and in the mental health consumer's clinical record, that the mental health consumer is medically fit to continue seclusion.
- If reasonable force in the form of physical restraint is needed to get the mental health consumer into the seclusion area and an oral or written authorisation has been made, then that is not a separate restraint event and a completion of a restraint form is not required.
- The person who authorised the seclusion must inform the mental health consumer's psychiatrist/duty psychiatrist of the seclusion within two (2) hours using the [Record of Information Medical Practitioner and Treating Psychiatrist of Seclusion Order \(Form 11C\)](#).
- The mental health consumer must be examined by a medical practitioner (not necessarily the practitioner who authorised the seclusion) within two (2) hours of seclusion commencing.
- Seclusion can be extended with documentation on the [Record of Examination of Secluded Person \(Form 11E\)](#) for periods of up to two (2) hours. An examination by a medical practitioner is always required before seclusion is extended
- Revoking or expiry of a seclusion order must be recorded on [Revocation or Expiry of Seclusion \(Form 11F\)](#).
- The most senior registered nurse on the ward (after hours the most senior registered nurse on duty), is to be informed of all seclusion events. This registered nurse will attend, sight the mental health consumer and ensure nursing practice and documentation comply with overarching policy, regulation and legislation.

Clinical practice consideration – restraint and seclusion:

If a person is restrained, and then a decision is made to also place the person in seclusion, then both a decision to restrain and a decision to seclude have been made and both forms need to be completed.

7. Patient care management – Seclusion event

Clinical practice consideration – sexual safety:

The following sexual safety considerations must be applied during any seclusion or restraint event in accordance with the Chief Psychiatrist's [Guidelines for the Sexual Safety of Consumers of Mental Health Services in WA](#)²:

- Mental health consumers should retain their own clothing unless there is a serious clinical contraindication which may compromise safety. Such reasons must be documented in the mental health consumer's healthcare record.
 - Mental health consumers must not be secluded naked or in underclothes.
 - Removing clothing, including underwear, is a last resort and is done on a case by case basis and that at the point of secluding a risk assessment is documented outlining the **current** risk being mitigated and steps taken to ensure client dignity is maintained (e.g. covering the client whilst they remove undergarments and allowing them to change into provided alternatives on their own once staff leave).
 - Items removed will be returned immediately upon cessation of seclusion or earlier if risk posed by these items is alleviated.
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- All hazardous items such as belts, shoes and laces, pens etc. must be removed.
 - To ensure staff safety, staff should never enter a seclusion room without sufficient staff numbers (minimum of three staff members).
 - While in seclusion, clinical observations must be performed by a mental health practitioner, registered nurse or enrolled nurse every 15 minutes, and must include monitoring and documentation of the mental health consumer's respiratory rate using the [Record of Observations Made of Secluded Person \(Form 11D\)](#). It is imperative that any change in the mental health consumer's physical condition, or sudden deterioration, is promptly recognised and reported – refer to the [Recognising and Responding to Acute Deterioration Policy](#).
 - The 15-minute clinical observations can be performed outside the room but must be direct observation, not through a closed-circuit television (CCTV) link (if available).
 - In addition to the 15-minute observations, CCTV (if available), should be activated once seclusion has commenced, with the mental health consumer observed continuously by a mental health practitioner or a registered or enrolled nurse.
 - The nurse in charge of the ward is to ensure that only necessary staff view the CCTV monitor throughout seclusion. The mental health consumer will be informed at the commencement of seclusion that CCTV facilities are used in the seclusion process.
 - If CCTV monitoring is not available, a Mental Health Practitioner or a registered or enrolled nurse must remain outside the seclusion room for the duration of the mental health consumer's seclusion.
 - Whilst caring for the mental health consumer in seclusion, the Mental Health Practitioner or a registered or enrolled nurse should be aware that this duty is a priority. In the event of an emergency, the allocated clinician must ensure that the mental health consumer remains under observation whilst in seclusion.
 - A medical practitioner must examine the person at least every two (2) hours and more frequently when the condition of the mental health consumer indicates more frequent medical intervention is appropriate.

- The purpose of the medical practitioner examination is to ensure that the mental health consumer's medical condition is satisfactory and to assess whether seclusion should continue or be revoked. Details of these examinations are to be documented on Form 11E.
- At any time, a medical practitioner, a mental health practitioner or the person in charge of the ward can revoke the seclusion order and the mental health consumer must be allowed to exit the seclusion area using Form 11F.
- A risk assessment must be conducted prior to a decision to revoke or the expiry of the seclusion order.
- Where a mental health consumer is in seclusion, it is essential to ensure that appropriate provision is made for their basic needs, including bedding, clothing, food, drink and toilet facilities.
- In accordance with the mental health consumer's consent and management plan, the family/carer or PSP will be informed as soon as practical of the use of seclusion and when the seclusion has been terminated.

8. Post seclusion management

Post incident support focuses on the immediate physical and emotional wellbeing of people involved in the incident. Post incident learning helps gain information to understand what happened and why, and to learn about how to improve care and support provided to the person involved. Together, post incident support and learning make up post incident debriefing. It is imperative that senior clinical and medical staff participate in the debriefing process.

- Following the release of a mental health consumer from seclusion, a medical practitioner will carry out a physical examination within six (6) hours to ensure that there is no complication of, or deterioration in, the mental health consumer's mental or physical condition that is a result of, or potential result of the mental health consumer being secluded using the [Record of Post Seclusion Examination \(Form 11G\)](#).
- The most senior registered nurse on the ward (after hours the most Senior Registered Nurse on duty) is responsible for ensuring the mental health consumer's risk assessment and management plan is reviewed and updated as part of the clinical review process in collaboration with the mental health consumer and/or their family/carer or PSPs, as soon as practicable post seclusion.
- Following seclusion, all mental health consumers must be offered support and counselling preferably from a person of their choice. Support may also include access to the Mental Health Advocacy Service.
- Aboriginal mental health staff should ensure the cultural welfare and safety of the mental health consumer following seclusion wherever possible
- A Post Seclusion Interview must be offered to the mental health consumer and is to be conducted as soon as practicable by a member of staff chosen by the mental health consumer. With the consent of the mental health consumer, Peer Support involvement and the mental health consumer's family/carer and/or PSP should be included in the review process.
- Mental health consumers have a right to refuse to engage in a Post Seclusion Interview, but every attempt should be made by staff to engage them in the process. If a mental health consumer chooses not to engage in a Post Seclusion Interview, this should be documented in the healthcare record.

- The information gathered by the Post Seclusion Interview is to be recorded on PAKMR216 which is to be stored in the mental health consumer's healthcare record. Following the Post Seclusion Interview a review of the mental health consumers Patient Safety Plan is to take place.

8.1 Staff debriefing

Line Managers must organise post restraint debriefing for staff in accordance with the [EMHS Post Incident Management Policy](#).

9. Documentation

The minimum documentation requirements for all seclusion events include:

- The relevant [MHA 2014 Forms](#) must be completed for every occasion of seclusion.
- All forms must be checked to ensure they are comprehensively and accurately completed and comply with the MHA 2014 legislative requirements prior to submitting to the Office of the Chief Psychiatrist (OCP).
- A copy of all seclusion forms must be forwarded to the Nurse Unit Manager-Mental Health or delegate.
- A copy of all seclusion forms will be provided to the OCP, and where applicable the Mentally Impaired Accused Review Board, after any seclusion order – refer to the [Flowchart for Seclusion Reporting to the Chief Psychiatrist](#).
- A copy of all the forms used in the seclusion process must be given to the mental health consumer unless it is determined by the medical officer that to do so may not be in the best interest of the mental health consumer or others. If the forms are not provided to the mental health consumer, the medical officer will document the reasons for this decision in the mental health consumer's healthcare record
- The mental health consumer's healthcare record must be contemporaneously updated to include:
 - Guidance on when to cease seclusion
 - Length of seclusion and any breaks in seclusion
 - The face-to-face medical examination of the mental health consumer in the 24 hours following seclusion.
 - Mental health consumer's post-seclusion presentation
 - Post seclusion review of the Management Plan which includes the review of the clinical observation level – refer to the [Safe and Supportive Observations Policy](#)
 - What action is required post-seclusion, including completion of necessary documentation
 - Identifiable factors which may have contributed to seclusion
 - Actions taken by staff prior to seclusion to defuse situation to avoid seclusion.
- PSOLIS alerts and incidents will be reviewed and updated.
- Datix CIMS online notification form must be completed whenever the seclusion has resulted, or may have resulted, in harm to the mental health consumer – refer to the [Clinical Incident Management Policy MP0122/19](#).
- If seclusion is preceded by a restraint, then the MHA 2014 Forms pertaining to the restraint are required to be completed in addition to seclusion documentation (see the [Mental Health Restraint Policy](#)).

- Transfer and discharge documentation should include triggers that resulted in seclusion and known individual de-escalation and preventative strategies.

10. Executive review of seclusion

An executive review of a seclusion by the Head of Department of Psychiatry, Coordinator of Nursing-Mental Health, and the Nurse Unit Manager must occur within seven days of the event occurring and outcome recorded via the [Seclusion and Restraint Evaluation Tool \(SecRET\)](#).

Where required, a formal case conference can be coordinated for complex cases, which may include:

- Mental health consumer who has three or more seclusion events within one (1) week
- Any seclusion longer than six (6) hours
- Any event which results in an injury to the mental health consumer, staff or others.

If required, the HoD Psychiatry can further escalate complex cases to the Service Director Mental Health for an independent review.

The HoD Psychiatry will ensure evidence of local interventions and reviews are available during the independent review.

11. Education and training requirements

The following principles are to be included for education and training programs regarding seclusion practices:

- Provision of staff access to, and attendance at, initial prevention and de-escalation training within six weeks of commencing work within the MHU, and annually thereafter in accordance with the [EMHS Mandatory Training Policy](#).
- Training is inclusive of the person with a lived experience of mental illness and family and supports
- A clear guide to ensure that staff know when an intervention is safe to implement and when to call for assistance. This may include a risk assessment for the potential for aggression and violence, but should focus on the appropriate verbal intervention for people experiencing agitation and anxiety that has not reached the point of acting out
- Understanding the theory of, and antecedents to, aggressive or challenging behaviours to improve prevention strategies and the use of least restrictive practices
- Include language and cultural considerations for Aboriginal people and people from CALD communities
- Include considerations for gender sensitive restrictive practices in accordance with the principles outlined in the Chief Psychiatrist's [Guidelines for the Sexual Safety of Consumers of Mental Health Services in WA](#)²
- Post incident debriefing including lived experience workers.

12. Roles and responsibilities

The following outlines the specific roles and responsibilities relevant to this policy:

Role	Responsibility
Head of Department	• Seclusion review within 7 days of event

Role	Responsibility
(HoD) - Psychiatry	• Further escalate complex cases to the Service Director Mental Health for an independent review
Managers and clinicians	• Ensure clinical staff are provided with continuing education and support in least restrictive practice and the use of restraint and seclusion • Ensure reduction in the use of restraint and seclusion and engage in monitoring of quality care and patient and population health outcomes to inform quality improvement • Ensure documentation and reporting requirements of the MHA 2014 and associated regulations are met • Provide support, debriefing and opportunities to learn • Aboriginal Mental Health Workers are to be involved following episodes of seclusion when it is appropriate to follow up the cultural welfare and safety of the patient
Medical staff	Medical staff must take a proactive role in minimising restrictive practices: • The treating or on-call Psychiatrist must take an active leadership role in facilitating strategies which reduce restrictive practices • Comply with the requirements of the MHA 2014 as they pertain to seclusion and restraint • Medical staff must take an active decision-making role early in the seclusion process to limit duration and maximise safety.
Security personnel	• Assist the clinical team under the direction of the Mental Health Practitioner or Medical Practitioner, or person in charge of the ward in accordance with this policy.

13. Compliance monitoring

Compliance against this policy will be monitored by the Comprehensive Care Steering Committee. Compliance monitoring activities relevant to this policy include:

- Episodes of seclusions and restraint (per 1000 bed days as denominator)
- Behaviour-related clinical incident trends (per 1000 bed days as denominator)
- API 3 training compliance rates
- Summary of findings and applicable improvement actions arising from Executive Review of Seclusion.

14. Legislative context

The following documents are required to give effect to this policy:

- *Mental Health Act 2014 (WA)*
- *Carers Recognition Act 2004 (WA)*

- [Chief Psychiatrist's Standards for Clinical Care | Chief Psychiatrist | Government of Western Australia](#)
- HDWA
 - [Consent to Treatment Policy](#)
 - [Clinical Incident Management Policy MP0122/19](#)
- EMHS
 - [Rights of Carers and other Personal Support Persons Policy](#)
 - [Discharge Against Medical Advice](#)
 - [Rights of Patients in Mental Health Policy](#)
 - [Use of Video Monitoring in Mental Health Inpatient Units Policy](#)
 - [Mandatory Training Policy](#)
 - [Post Incident Management / Critical Incident Stress Debriefing Policy](#)
 - [Reducing Restrictive Practices in Mental Health Services Policy](#)
 - [EMHS Discharge Against Medical Advice Policy.](#)
- AKG
 - [Recognising and Responding to Acute Deterioration Policy.](#)
 - [Consumer Safety Planning Procedure](#)

15. Supporting information

The following documents support the implementation of this policy:

- AKG
 - [Safe and Supportive Observations Policy](#)
 - [Mental Health Restraint Policy](#)
 - [Patient Safety Plan Procedure](#)
- Other
 - [Guidelines for the Sexual Safety of Consumers of Mental Health Services in WA](#)

[Mental Health Act 2014 Forms](#) applicable to this policy:

- Form 11A – Record of oral authorisation of seclusion
- Form 11B – Written seclusion order
- Form 11C – Record of information medical practitioner and treating psychiatrist of seclusion order
- Form 11D – Record of observations made of secluded person
- Form 11E – Record of examination of secluded person
- Form 11F – Revocation or expiry of seclusion
- Form 11G – Record of post seclusion examination

16. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2 – Action 2.3
- Standard 2 – Action 2.4
- Standard 5 – Action 5.33
- Standard 5 – Action 5.34
- Standard 5 – Action 5.35
- Standard 5 – Action 5.36

National Standards for Mental Health Services (2010):

- Standard 1 – Criteria 1.9

17. References

1. Office of the Chief Psychiatrist WA 2022. Chief Psychiatrist's Standards for Clinical Care. [online] Available at: <https://www.chiefpsychiatrist.wa.gov.au/standards-guidelines/chief-psychiatrists-standards-for-clinical-care> [Accessed 8 April 2022]
2. Office of the Chief Psychiatrist WA 2020. Chief Psychiatrist's Guidelines for the Sexual Safety of Consumers of Mental Health Services in Western Australia. [online] Available at: <https://www.chiefpsychiatrist.wa.gov.au/wp-content/uploads/2020/12/Chief-Psychiatrists-Sexual-Safety-Guidelines-2020.pdf> [Accessed 30 June 2022]
3. Melbourne Social Equity Institute (2014) Seclusion and Restraint Project: Report, Melbourne: University of Melbourne

18. Glossary

The following definitions are relevant to this policy.

Term	Definition
MHU	Mental Health Unit
OCP	Office of Chief Psychiatrist
PSP	Personal Support Person
Restrictive interventions	Interventions that may infringe a person's human rights and freedom of movement, including observation, seclusion, manual restraint, mechanical restraint and rapid tranquillisation.
Seclusion	Confinement of a person, who is being provided with treatment or care at an authorised hospital, by leaving the person at any time of the day or night alone in a room or area from which it is not within the person's control to leave.

19. Policy owner

Service Director Mental Health

Enquiries relating to this policy may be directed to:

Title: Coordinator of Nursing - Mental Health
 Department: Mental Health Service
 Email: Dene.Olive@health.wa.gov.au

20. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V2	12 11 2025	12 11 2028	Full review, minor amendments only.

21. Authorisation

This policy has been authorised and issued by:

Authorised by	Karen Elliott - Service Director Mental Health
Authorisation date	11 11 2025
Published date	12 11 2025